

CAGRILINTIDE RESEARCH PROTOCOL — ONE PAGER

10 mg Vial + 2 mL Bacteriostatic Water | Concentration: 5 mg/mL | Educational / Research Reference Only

1. RECONSTITUTION & CONCENTRATION

Vial: 10 mg lyophilized cagrilintide **Diluent:** 2.0 mL bacteriostatic water (0.9% benzyl alcohol) **Final concentration:** 5.0 mg/mL.

Unit conversion (U-100 syringe): 1 unit = 0.01 mL = **50 mcg (0.05 mg)**. 0.25 mg = 5 u | 0.5 mg = 10 u | 1.0 mg = 20 u | 1.7 mg = 34 u | 2.4 mg = 48 u | 4.5 mg = 90 u.

Steps: Alcohol-swab tops → draw 2.0 mL BAC water → inject slowly down inner glass wall (never onto powder) → gently swirl/roll until clear (never shake) → label date + “5 mg/mL” → refrigerate immediately at 2–8 °C, protected from light.

2. TYPICAL TITRATION SCHEDULE (ONCE-WEEKLY SUBCUTANEOUS)

Aligned with clinical trial patterns. Start low; escalate slowly to manage GI side effects (nausea most common). Hold or reduce a step if needed. Same day each week preferred.

Phase / Weeks	Weekly Dose	Units	Volume	Notes
Weeks 1–4	0.25 mg	5 units	0.05 mL	Initiation / GI adaptation
Weeks 5–8	0.50 mg	10 units	0.10 mL	First escalation
Weeks 9–12	1.0 mg	20 units	0.20 mL	Intermediate
Weeks 13–16	1.7 mg	34 units	0.34 mL	Approaching target
Week 17+	2.4 mg (maint.)	48 units	0.48 mL	Common later-trial target
Optional higher*	Up to 4.5 mg	Up to 90 u	Up to 0.90 mL	Phase 2 monotherapy only

*Higher doses studied in Phase 2 monotherapy; most later development centers on 2.4 mg. Prioritize tolerability.

3. INJECTION FREQUENCY, VIAL DURATION & CYCLE LENGTH

Frequency: Once weekly SC (abdomen / thigh / upper arm; rotate sites). Half-life ~7–8 days supports weekly dosing.

Peptide content vs. stability: At the slow titration above, one 10 mg vial supplies enough peptide for ~13 weeks (~8.7 mg used through Week 13). However, once reconstituted the practical limit is governed by stability (see Section 4). At 2.4 mg maintenance a vial yields ~4 doses. Clinical trials ran 26–68 weeks continuous; research protocols often target at least 16–20 weeks to complete titration and observe effect.

4. RECONSTITUTED STABILITY — IMPORTANT NUANCE

The commonly cited ~28-day window after reconstitution is primarily a **conservative sterility / preservative guideline**, not proven evidence that the cagrilintide molecule itself undergoes sharp chemical degradation at day 28 under ideal refrigeration.

- **Source of the 28-day figure:** Manufacturer labeling for bacteriostatic water (discard 28 days after first puncture) plus general research-peptide convention. Microbial risk rises with repeated needle entries even though benzyl alcohol inhibits growth.
- **Cagrilintide-specific caution:** As an amylin analogue it has a higher intrinsic tendency to form fibrils/aggregates than pure GLP-1-family peptides. Optimal pH is often acidic (~3.5–4.5). This is why many sources stay more cautious (14–28 days) compared with retatrutide or semaglutide.
- **Comparison to retatrutide:** Many users successfully run larger reconstituted retatrutide vials 6–12+ weeks with little apparent loss of effect when technique and refrigeration are good. Community and some third-party testing reports support this for Reta. Cagrilintide warrants relatively more caution because of aggregation risk.
- **Practical recommendation:** Keep reconstituted solution at 2–8 °C, protected from light, never freeze. Inspect for cloudiness or particles before every draw and discard if present. Prefer using a reconstituted vial within 4–6 weeks when possible. Extending further is common in research settings but accepts higher uncertainty on potency and sterility. Lyophilized powder remains the stable long-term form (preferably –20 °C).

5. ADDITIONAL NOTES

Cagrilintide is investigational (not FDA-approved). Primary observed effects: reduced appetite, slowed gastric emptying, dose-dependent weight loss. Common AEs: GI (nausea, etc.). Use sterile technique and proper sharps disposal only.

DISCLAIMER: Compiled from clinical-trial publications (e.g., Lau et al. Lancet 2021; CagriSema/REDEFINE summaries) and research-peptide educational sources for a 10 mg vial + 2 mL BAC water. The 28-day stability figure is a conservative guideline, not a proven chemical cliff. This document is for educational/research reference only and is not medical advice. Consult a qualified healthcare professional. Individual response and professional oversight determine actual use. Verify product quality and current regulatory status of any research material.